Maplewood General Hospital

400 Orchard Street, New Haven, CT 06511 | Main line (203) 555-0100

Hospital Discharge Summary and Patient Instructions

Patient	Chen Guoliang	**MRN**	MGH-0057321
Date of birth	01/30/1947	**Unit**	Neurology 3 West
Admitted	10/18/2026	**Discharged**	10/21/2026
Attending	Rachel Kim, MD (Neurology)	**Preferred language**	Mandarin Chinese (interpreter used; daughter reads sir

Why you were in the hospital

Diagnosis: Transient ischemic attack (TIA, a "mini-stroke") caused by new atrial fibrillation (an irregular heartbeat that can form clots). Other conditions: type 2 diabetes, high blood pressure.

You had 20 minutes of right arm weakness and trouble speaking that went away fully. Your brain MRI showed no permanent damage. We started a blood thinner to prevent a stroke.

Hospital course (for your care team)

79M with T2DM, HTN, CKD 3a (eGFR 48) presenting with transient R arm weakness and expressive aphasia lasting 20 min, NIHSS 0 on arrival. MRI brain without acute infarct; CTA head/neck without significant stenosis. Telemetry with new paroxysmal AF, CHA2DS2-VASc 6. Started apixaban 5 mg BID (dose-reduction criteria not met). Aspirin discontinued. High-intensity statin. Metoprolol for rate control. Metformin reduced given eGFR. Patient takes OTC ginkgo biloba, advised to stop. 14-day ambulatory ECG monitor ordered. Mandarin interpreter used for all teaching; daughter present.

Your medications

START taking these medications

Apixaban (Eliquis) 5 mg tab: 1 tab PO BID to prevent stroke. Do not skip doses or stop unless your doctor tells you to.

Metoprolol tartrate 25 mg tab: 1 tab PO BID to control heart rate.

CHANGE how you take these medications

Atorvastatin 80 mg tab: 1 tab PO QHS. Dose increased from 20 mg.

Metformin 500 mg tab: 1 tab PO BID with breakfast and dinner. Dose lowered from 1,000 mg because of kidney function.

CONTINUE taking these medications

Amlodipine 5 mg tab: 1 tab PO daily for blood pressure.

Acetaminophen (Tylenol) 500 mg tab: 1-2 tabs PO q6h PRN headache or pain. Max 3,000 mg/day.

STOP taking these medications

Aspirin 81 mg tab: STOP. You are now on apixaban; taking both raises bleeding risk.

Ginkgo biloba (supplement): STOP. It can increase bleeding with apixaban.

Diet and activity

Check your blood pressure and pulse every morning with the home monitor and write them down. Bring the log to your visits.

Do not drive until cleared by your neurologist. Low-salt, diabetic diet. Walk daily as tolerated.

Follow-up appointments and tests

Blood test (kidney function): Wednesday 10/28/2026, between 7:00 AM and 5:00 PM, Maplewood Outpatient Lab. No appointment needed.

Neurology clinic, Dr. Rachel Kim: Wednesday 11/04/2026 at 9:00 AM, Maplewood Neuroscience Center, 4th floor. A Mandarin interpreter has been booked.

Cardiology clinic, Dr. Samuel Osei: Tuesday 11/10/2026 at 2:15 PM, Heart and Vascular Center, 2nd floor. Bring your blood pressure log.

14-day heart monitor: the monitoring company will mail it to your home and call you with instructions.

Primary care: see your primary care doctor within 1 week. Call (203) 555-0130 to schedule.

When to get help

Call 911 right away (BE FAST) if: face drooping on one side; arm or leg weakness; trouble speaking or understanding; sudden loss of vision; sudden loss of balance; sudden severe headache; vomiting blood or black, tarry stools.

Call the Neurology Clinic at (203) 555-0150 if: a nosebleed or bleeding gums do not stop after 10 minutes; your urine turns pink or red; you have unusual bruising; your pulse is below 50 or above 120 on the home monitor.